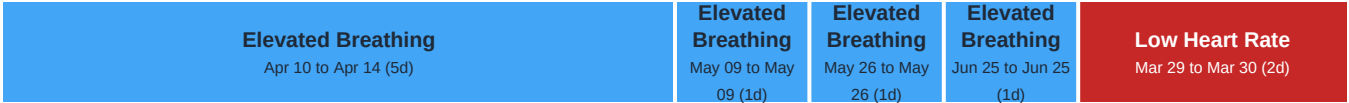


Patient ID: S (Bed)Period: March 27 to June 25, 2024Report Date: June 26, 2024Coverage: 547/2150h (25%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 91 days from Mar 27 to Jun 25

■ HR ■ Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	66 bpm	65 bpm	66 bpm
Breathing	26 brpm	26 brpm	27 brpm

Limited data in the final 24h (1/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode in the morning, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jun 25	11 AM to 12 PM	1h	1	Elevated Breathing	26	26	27	Check SpO2 and lung sounds; assess for fluid overload or early infection

8 episodic events Detected, spanning 9h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Skip

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
8	9h	Low Heart Rate	<1	25%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 29	9 PM to 11 PM	3h	2	Low Heart Rate	43	40	69	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 10	8 AM to 9 AM	3h	3	Elevated Breathing	26	10	34	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 09	8 PM to 9 PM	1h	1	Elevated Breathing	28	27	28	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 26	8 PM to 9 PM	1h	1	Elevated Breathing	24	17	28	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jun 25	11 AM to 12 PM	1h	1	Elevated Breathing	26	26	27	Check SpO2 and lung sounds; assess for fluid overload or early infection

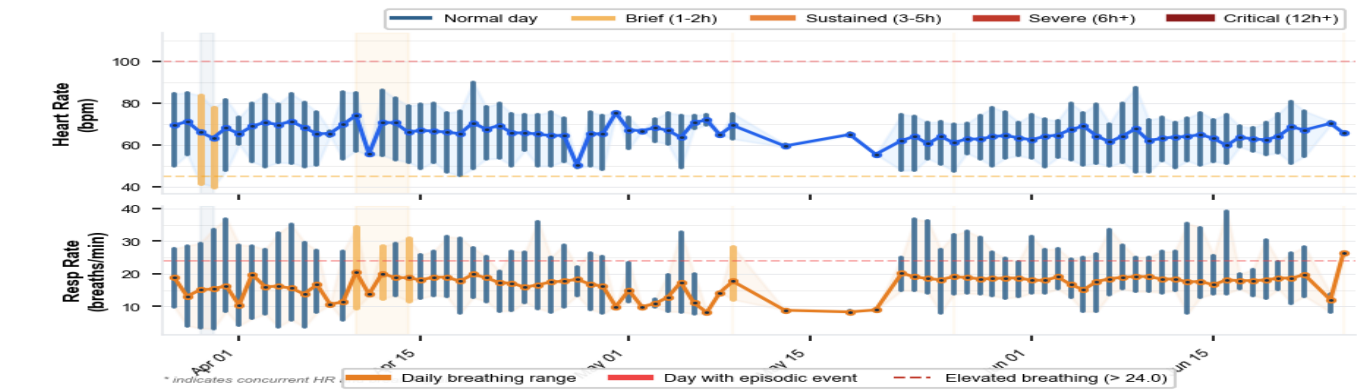
Insufficient data coverage (25%, below 30%).

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 34 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 5 unstable phases with 16 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

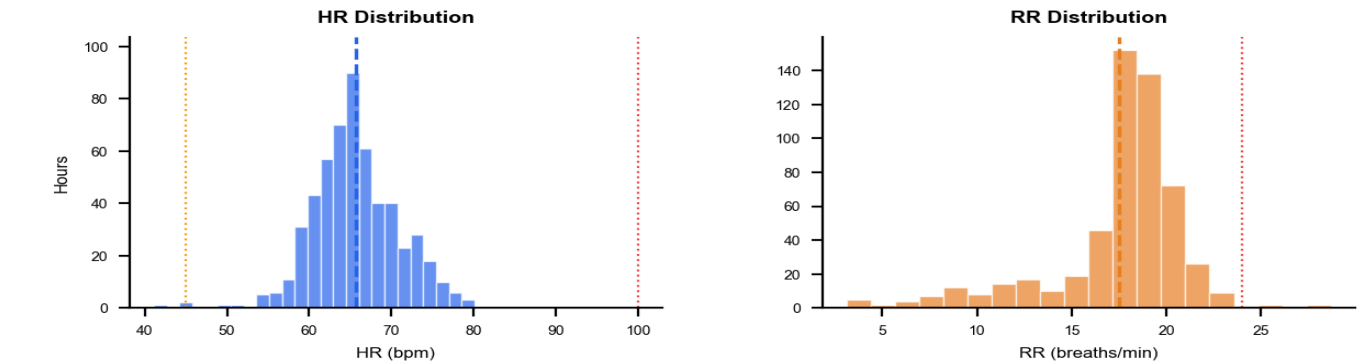
Clinical Pattern Observations:

- 1. **Breathing variability:** RR spread 12 brpm (8 to 21).
- 2. **Clustered pattern:** Episodic events on 6 of 91 days (6%).
- 3. **Nocturnal pattern:** 5 of 6 eligible episodes during evening or night hours.

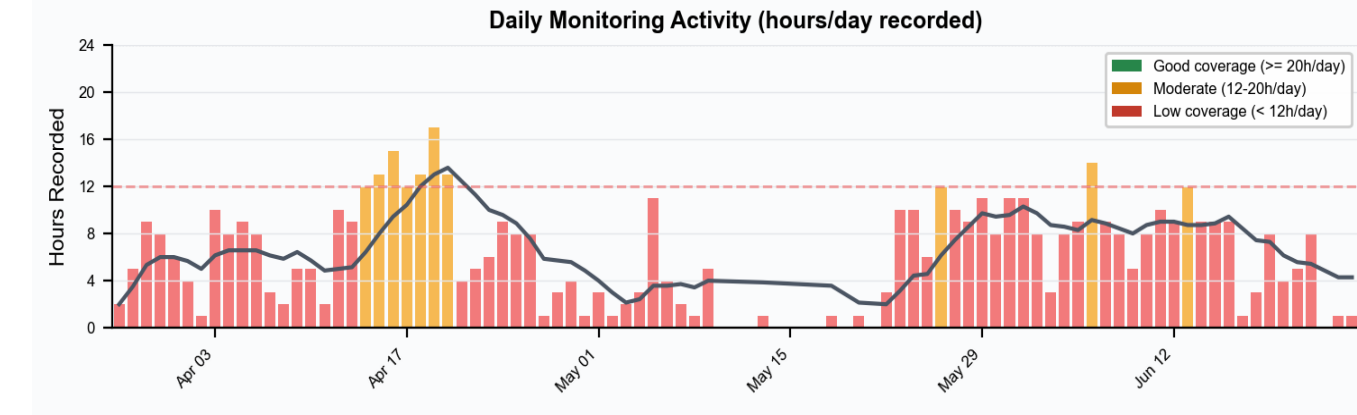


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.